

CH SHARIF AND SAEED HOSPITAL

Hospital Management System

+ Standalone Pharmacy Integration

CLIENT FUNCTIONAL WALKTHROUGH & SOLUTION BLUEPRINT

A complete client-facing explanation of portals, patient journeys, appointments, billing, admission, pharmacy, inventory ledgers, cash settlement, Staff 360° profiles, biometric attendance, salary payroll, doctor commission, management reporting and future staff self-service integration.

Document Basis

Prepared from the approved Final Source of Truth v6.3. This full client edition includes all latest ownership, ledger, Staff 360°, payroll, commission, appointment, admission, cash-settlement and Pharmacy Manager inventory decisions.

Prepared for CH Sharif and Saeed Hospital

Prepared by iSysware Software Solutions

1. Executive Summary

What the solution is designed to achieve

The proposed solution is a connected hospital operations platform built around clear ownership: each portal performs the work it is responsible for, financial hand-offs are traceable, and the standalone Pharmacy remains integrated without duplicating medicine stock or Pharmacy billing inside the HMS.

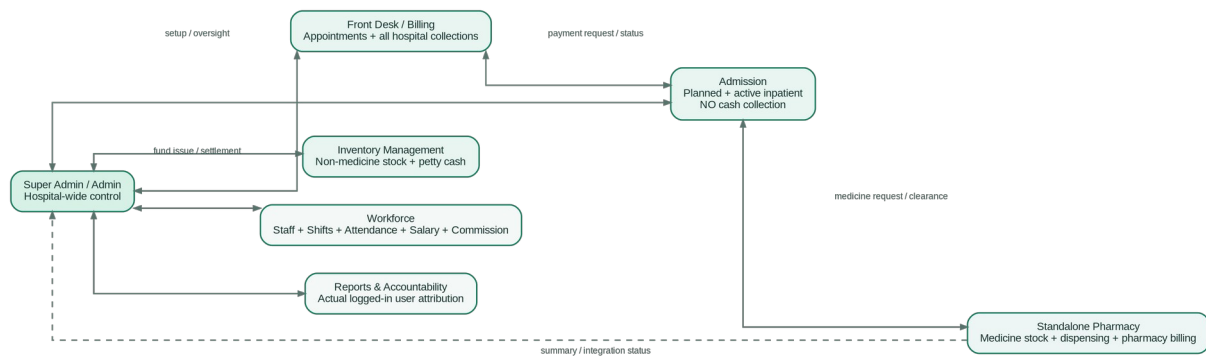
- Five HMS portals: Super Admin, Admin, Front Desk / Billing, Admission, and Inventory Management.
- A separate Pharmacy project with Pharmacy Super Admin, Pharmacy Manager, and Pharmacy Sales / Dispensing Staff portals.
- Front Desk / Billing is the only HMS portal that receives hospital patient money, including appointment advances and admission partial/final payments.
- Admission manages inpatient operations but does not collect cash. It sends payment requests to Billing and medicine requests to Pharmacy.
- Every authorized cash-handling user has an individual Balance Sheet and submits their own Account Settlement.
- Staff HR records and portal login accounts are separate. Every employee has a Staff 360° profile; portal access is only linked when required.
- Attendance flows from a biometric/device adapter to reviewed/approved attendance; Super Admin/Admin can correct it with full change history before Salary Payroll.
- Salary Payroll and Doctor Commission are completely independent generation, approval, payout, outstanding and reporting streams.
- Only Panel Patients are reusable permanent patient masters; normal/self-pay identities are temporary case/booking identities while invoices and transaction history remain available.

Core Design Principle

Every operational and financial action is attributed to the actual logged-in user: Collected By, Purchased By, Approved By, Dispensed By, Paid By, Settled By, Changed By, and similar accountability fields.

2. System Scope at a Glance

The business boundary between HMS and Pharmacy



Overall portal and system connection map

Portal / System	Owns	Cash Role
HMS Super Admin	Hospital-wide control, setup, workforce, finance, reports	Receives/reviews settlements; may hold management cash if assigned
HMS Admin	Same normal management functions as Super Admin	Receives/reviews settlements; protected Super Admin accounts cannot be altered
Front Desk / Billing	Appointments, OPD/OBS/ER intake, hospital invoices, all hospital patient collections	Yes - own user Balance Sheet + own settlement
Admission	Planned admissions, active inpatient stay, bed/services, payment and Pharmacy coordination	No hospital cash collection
Inventory Management	Non-medicine stock, suppliers, purchases, department issues	Yes when petty cash/advance is assigned
Standalone Pharmacy	Medicine stock, dispensing, Pharmacy invoices, Pharmacy cashier settlement	Yes for authorized Pharmacy cash users

3. Staff, Portal Users and Access

HR identity is not the same as a system login

Staff Master

Staff Master is the hospital workforce record for doctors, nurses, technicians, reception/billing staff, admission staff, inventory staff, peons, ward/support staff and other employees. A staff member does not automatically receive a portal login.

Staff 360° profile area	What Super Admin / Admin can view
Overview	Employee ID, category, department, designation, contact, joining date and employment status.
Employment / Shift	Current and historical department, designation and shift assignments with effective dates.
Attendance	Daily/period attendance, biometric mapping, late/early/absence, correction history and reasons.
Salary	Staff-specific salary basis/rate, deduction rules, generated salary, paid/partial/outstanding and salary slips.
Commission	Where applicable: staff-specific service commission rules, accrual, generated statements, paid/partial/outstanding.
Portal Access	Optional linked Portal User ID/status; a staff record can exist with No Portal Access.
Future Staff Portal	The same staffId can later expose only the employee's own profile, attendance, salary slips and commission statements.

Portal User

A Portal User is an authentication and access record. It contains credentials, portal assignment and account status. Operational users can be linked to a Staff record through staffId, but access credentials remain separate from HR, attendance, salary and commission data.

Example

A Peon can have a Staff record with attendance and salary but no login. A Billing Officer can have both a Staff record and a linked Front Desk / Billing portal account.

Management protection

Super Admin and Admin have the same normal hospital-management functionality. The protected difference is governance: Admin cannot create, modify, deactivate, delete, reset credentials, demote or otherwise alter a protected Super Admin account.

4. Super Admin & Admin Portals

Hospital-wide control with protected root governance

Management Area	What the client can do
Dashboard & Hospital Overview	View operational, financial and attention indicators across the hospital.
Hospital Setup	Manage departments, services/rates, wards, rooms and beds.
Users & Staff	Manage Admin users, operational portal users, Staff Master, individual Staff 360° profiles, employment/shift history and attendance.
Patient / Panel	Manage Panel Patients, corporate/panel setup and service discount rules.
Appointments & Admissions	Monitor appointments, planned admissions, bed status and financial/clearance progress.
Finance & Cash Control	Billing overview, collections, refunds, discounts, outstanding, expenses, user balances, petty cash/advances and settlements.
Payroll	Generate, approve, pay and report Salary Payroll.
Doctor Commission	Generate, approve, pay and report Doctor Commission separately from salary.
Inventory & Pharmacy Oversight	Monitor inventory operations and Pharmacy integration summaries/status.
Reports	Hospital-wide plus individual staff reports: attendance, salary, commission, shift/employment, user activity and period-based operational/financial reports with export.

Same operations, one protected difference

Admin works with the same hospital operational modules as Super Admin, but cannot alter protected Super Admin accounts or use Admin credentials to enter the Super Admin portal.

5. Hospital Setup & Master Data

The configuration foundation for all operational flows

Before daily operations, management configures the shared masters used by Front Desk, Admission, Inventory, Payroll and reporting. Shared setup never forces one salary or commission rule on all employees: actual compensation and deduction values are assigned on each Staff profile.

Master	Purpose
Hospital Profile	Official identity used in invoices, receipts and exports. Unconfigured details remain "Not configured" rather than fabricated.
Departments	Clinical and administrative departments; wards are not treated as departments.
Services & Rates	Chargeable services, billing units, standard rates, panel eligibility, discount/override settings.
Wards / Rooms / Beds	Department -> Ward -> Room -> Bed hierarchy with operational and occupancy states separated.
Panels	Panel organizations, eligible patient links and service-level discount rules.
Shifts & Attendance Policies	Department-wise shifts, start/end times, grace rules, late/early exit treatment.
Shared Workforce Defaults / Templates	Shared salary types, deduction methods, commission methods and policy templates only. Actual salary rate, deductions and doctor service-commission rules are assigned per Staff profile during Add/Edit Staff.

6. Patient Identity & Retention Model

Permanent Panel Patients; temporary self-pay identities

Patient Type	How the system treats the identity	What remains after closure
Panel Patient	Reusable permanent patient master linked to panel membership and discount rules.	Patient master + transaction/encounter history.
Normal / Self-Pay OPD	Temporary booking/visit identity used for the current encounter.	Invoice, receipt and transaction snapshot remain; no reusable permanent master.
Normal / Self-Pay Admission	Temporary active admission identity for the stay.	Admission references, hospital invoice, Pharmacy references, receipts and discharge snapshot remain.

Why this matters

The system keeps financial and operational history without turning every self-pay visitor into a permanent reusable Patient Registry record. This is a deliberate CHSS business rule.

7. Front Desk Appointment Booking

Advance collection, check-in and final OPD billing



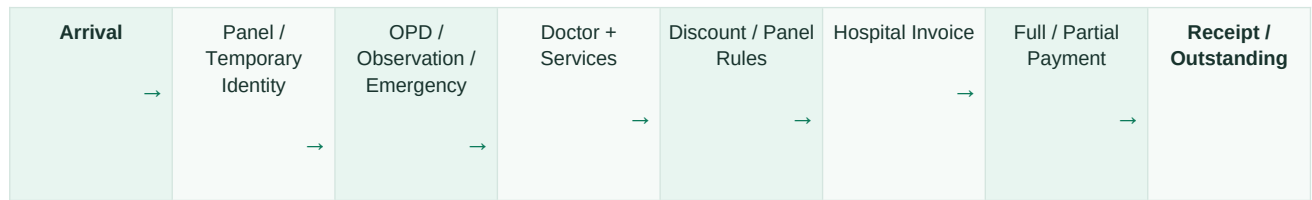
Appointment / walk-in to encounter completion

1. Front Desk creates a new appointment using a permanent Panel Patient or a temporary self-pay booking identity.
2. The user selects department, doctor, available slot and consultation/service.
3. The appointment can remain Unpaid, be Partially Paid, or have the estimated amount covered in advance.
4. Any advance is collected by Front Desk / Billing and a receipt is generated against the exact Billing user.
5. At check-in, the appointment converts to the actual OPD / Observation / Emergency encounter.
6. Final services are billed, the advance is adjusted, and Billing collects any remaining balance.
7. Reschedule, cancellation and no-show retain traceability; refund/credit follows configured policy.

Appointment Status	Meaning
Draft / Booked	Booking exists; may not yet be financially confirmed.
Confirmed	Appointment confirmed for the selected slot.
Checked-In	Patient arrived; encounter can start.
Completed	Encounter completed and final billing handled.
Rescheduled / Cancelled / No Show	Lifecycle exception retained for reporting.

8. Walk-in OPD / Observation / Emergency Billing

One intake portal, multiple encounter types



Observation and Emergency are encounter/workflow types, not separate login portals. Front Desk creates the case and performs hospital billing; the relevant clinical department performs the clinical work.

Discount and Doctor share

The default calculation is Gross Service Amount -> Discount -> Net Eligible Service Amount -> Doctor Commission -> Hospital Remaining Share. By default, Doctor Commission is calculated on the net amount after discount. A doctor contract may optionally use gross amount as the commission base.

Example	Amount
Gross service	PKR 5,000
10% discount	- PKR 500
Net eligible service	PKR 4,500
Doctor commission @ 20% of net	PKR 900
Hospital remaining share	PKR 3,600

9. Planned Admission / Admission Booking

Booking and advance coordination before the patient arrives

1. Doctor recommends admission and Admission creates a Planned Admission record.
2. The patient is identified as Panel or temporary self-pay.
3. Admission records expected date/time, department, doctor, ward/room/bed preference and estimated hospital amount.
4. If an advance is required, Admission sends a payment request to Front Desk / Billing.
5. Billing collects full or partial advance and issues the receipt; Admission only reads the payment status.
6. On arrival, Admission verifies the booking, assigns the actual bed and converts the booking into an Active Admission.

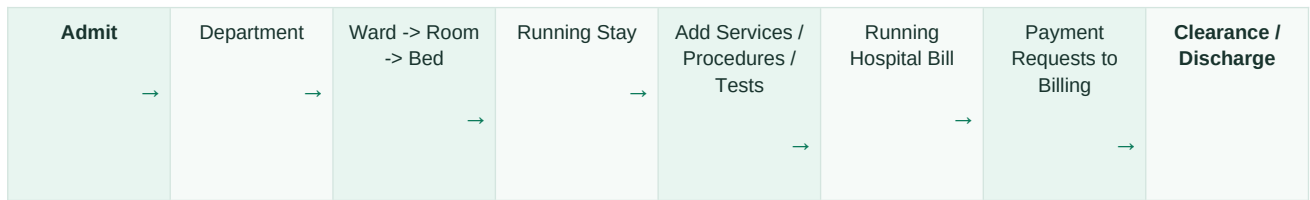
Important payment rule

Admission never physically receives hospital money. Even planned-admission advances, partial inpatient payments and final Hospital Bill payments are collected in Front Desk / Billing.

A booking may tentatively reserve a bed if hospital policy allows, but the bed becomes Occupied only when the Active Admission starts.

10. Active Admission Lifecycle

Running hospital services, bed state and payment requests



During the stay, Admission records hospital-side services, procedures, tests, bed charges and transfers. The running Hospital Bill is visible for coordination, but cash collection remains with Billing.

Partial hospital payment

When payment is needed, Admission sends a payment request. Billing collects any partial amount, posts the receipt to the Hospital Bill and returns the updated paid/outstanding status to Admission. This can repeat multiple times during the stay.

Bed and transfer controls

- Bed is Occupied only after admission activation.
- Transfers store From Bed, To Bed, timestamp and actual user.
- Final discharge releases the bed after required clearances are complete.

11. Medication Fulfillment During Admission

SELF vs HOSPITAL MANAGED - changeable with history

Mode	Admission behavior	Pharmacy / Billing effect
SELF	Record that patient/attendant will arrange medicines externally.	No Pharmacy request and no Pharmacy invoice for those self-sourced medicines.
HOSPITAL MANAGED	Admission creates medicine requests and tracks fulfillment.	Standalone Pharmacy Accepts / Partially Fulfills / Rejects, dispenses, bills and sends clearance.

Mode can change during the stay

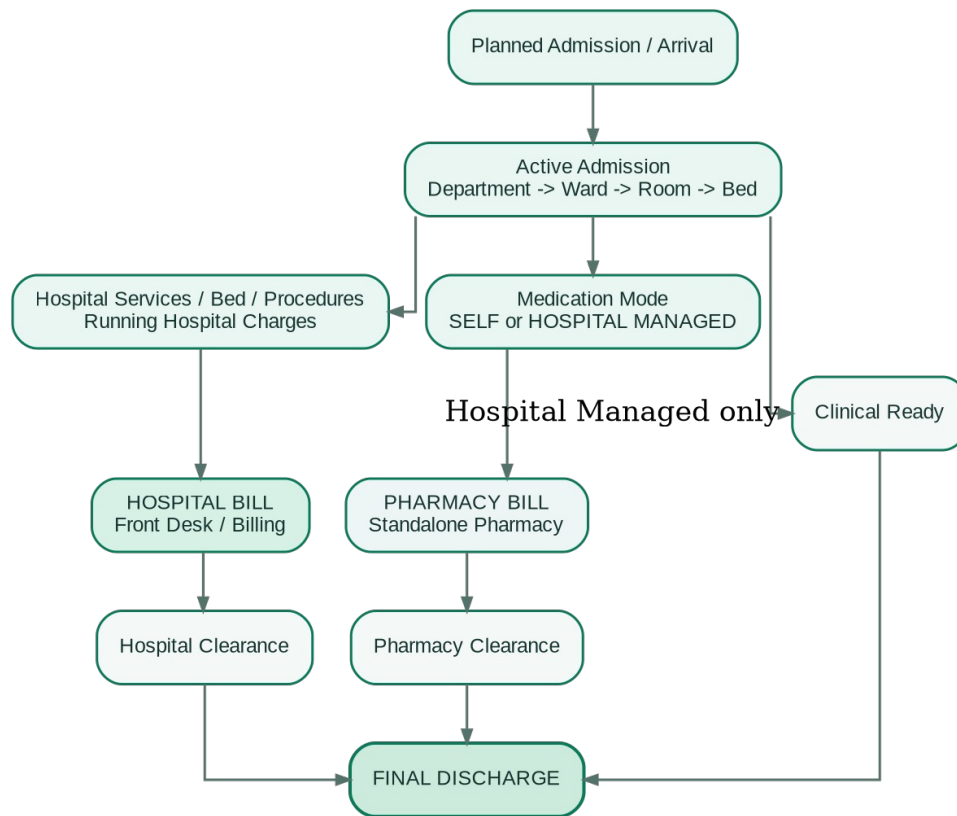
Admission can switch SELF -> HOSPITAL MANAGED or HOSPITAL MANAGED -> SELF. Every change stores Reason, Changed By and timestamp. When changing to SELF, only pending unfulfilled request lines may be cancelled. Already dispensed medicines remain billed unless a proper Pharmacy return/refund reverses them.

No silent deletion

A mode change changes the future fulfillment route; it does not erase previously dispensed Pharmacy transactions.

12. Admission Billing & Final Discharge

Two independent bills with coordinated clearance



Admission produces a Hospital Bill and a separate Pharmacy Bill

Financial Stream	Owner	Payment / Clearance
Hospital services, bed, tests, procedures	HMS / Front Desk Billing	Front Desk collects all advances, partials and final balance; grants Hospital Clearance.
Medicines dispensed by hospital-managed Pharmacy	Standalone Pharmacy	Pharmacy generates its own invoice/payment status and grants Pharmacy Clearance.

Final discharge gate

Clinical Ready + Hospital Clearance + Pharmacy Clearance = Final Discharge. A consolidated discharge summary may show both totals, but it is not a third accounting invoice.

13. Standalone Pharmacy System

Separate project, connected to HMS through controlled hand-offs

Pharmacy Portal	Primary responsibilities
Pharmacy Super Admin	Users, settings, medicine master, stock governance, finance, cash settlement oversight, reports and HMS integration monitoring.
Pharmacy Manager	Medicine master; suppliers; purchase/receipt; batches/expiry/cost; Medicine Stock Ledger; Supplier Ledger; store/counter transfers; patient/supplier returns; adjustments; FEFO; low-stock/near-expiry/expired alerts; HMS request review; receiving and staff supervision.
Pharmacy Sales / Dispensing Staff	Retail sales, HMS-linked dispensing, Pharmacy invoices, permitted returns/refunds, own Balance Sheet and own Account Settlement.

HMS-linked medicine request



- Pharmacy is the source of truth for medicine catalogue, batches, expiry, medicine stock, dispensing and Pharmacy invoices.
- HMS does not duplicate medicine stock ownership.
- Every request carries stable Admission ID, Medicine Request ID and case/patient reference.
- Duplicate callbacks must be idempotent; one dispense must not create two charges.
- Failed integration states remain visible and retryable rather than being silently marked complete.

14. Universal User Cash Architecture

Petty cash and settlement are not Inventory-only



Every authorized cash-handling user settles their own cash position

Cash-holding user	Typical money in	Typical money out
Front Desk / Billing user	Opening float/petty cash, cash patient collections, admission advances/payments	Cash refunds, authorized cash expenses
Inventory user	Opening petty cash, purchase advance/top-up, cash return/refund received	Cash purchases, approved operating expense
Pharmacy Sales / Dispensing user	Opening float/petty cash, cash retail/HMS Pharmacy collections	Cash refunds, authorized cash expense
Management cash-holder (optional)	Settlements received, assigned management cash	Petty-cash issues and approved disbursements

Physical vs online money

Online, card and bank receipts are reconciled separately and do not increase the user's physical cash balance.

15. Account Settlement

Every cash user closes their own account

Each authorized cash user has a “My Balance Sheet” and “My Account Settlement”. The system calculates expected cash from that user’s own unsettled transactions, compares it with the physical amount counted/submitted, and stores any variance.

Settlement Stage	What happens
Prepare	User selects the unsettled day / shift / custom period and reviews inflows/outflows.
Count	User records physical cash and the system calculates variance against expected cash.
Submit	Exact logged-in user submits their own settlement. One user cannot submit another user’s money as their own.
Review	Authorized reviewer verifies expected cash, physical cash, included transactions and remarks.
Decision	Accept / Partial Accept / Return / Reject.
Transfer	Accepted amount transfers custody to the receiver; unaccepted balance remains with the original user.

- A settlement is not revenue or expense; it is a transfer of custody of existing cash.
- Already-settled transactions cannot be included in another settlement.
- Partial settlement keeps remaining balance open for the same user.
- Each settlement keeps Submitted By, Received By, Reviewed By, timestamps and remarks.

16. Inventory Management

Non-medicine purchasing, stock and cash accountability



Inventory requirement to purchase and settlement

Funding & purchase

If the Inventory user does not have enough assigned cash for a required purchase, the user sends a Fund Request. Admin/Super Admin approves or rejects it. Approved petty cash/advance is credited to that exact Inventory user. Purchase/Receipt posts stock into the Stock Ledger and, when applicable, posts purchase/payment/outstanding into the Supplier Ledger. Cash payment reduces that user's Balance Sheet.

Purchase captures	Rule
Supplier	Supplier master stores contact, terms/status and purchase history; supplier ledger is linked for purchases, returns, payments, credits and outstanding when credit is used.
Product / Quantity / Rate	Used to calculate and receive stock.
Invoice / Reference	Links the transaction to the source document/reference.
Payment Method	Petty Cash reduces user physical cash; Management Direct/Bank does not.
Stock Ledger	Every purchase receipt, issue, return, transfer and adjustment posts a dated IN/OUT movement with running quantity.
Supplier Ledger	Tracks purchases, supplier returns, supplier payments, credits and outstanding balance where credit purchases are allowed.

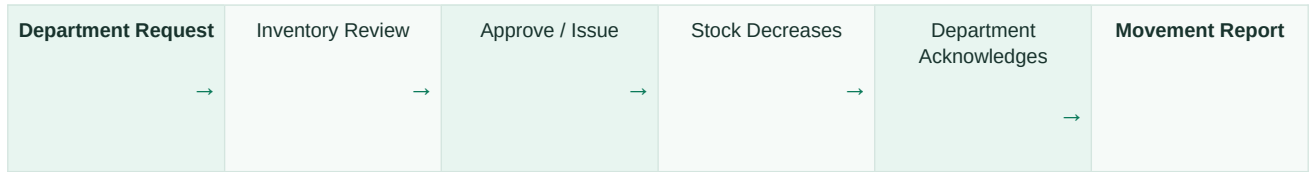
Supplier + Stock Ledgers Included

HMS Inventory now maintains both a Stock Ledger and a Supplier Ledger. Supplier outstanding is tracked when purchases are on credit; cash purchases still reduce the exact Inventory user's assigned cash.

17. Inventory Stock Issues, Returns & Reports

Controlled movement after purchasing

Department requisition / stock issue



Purchase return

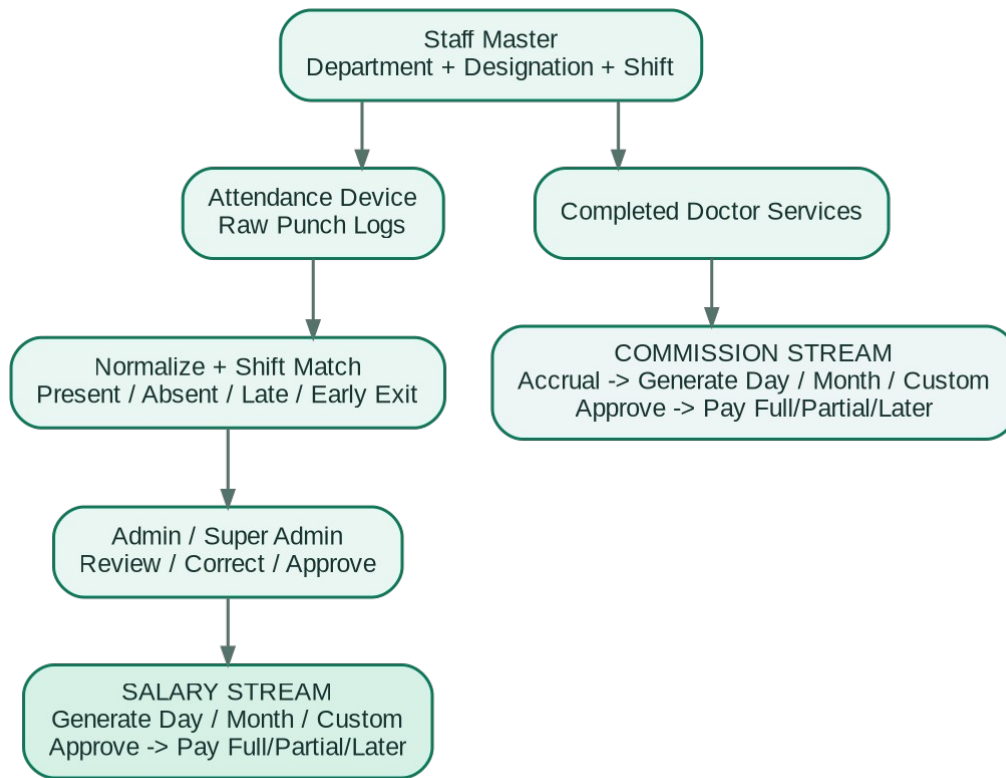
A purchase return links to the original purchase/item, records returned quantity and supplier/refund method, decreases stock, and updates cash correctly. If cash is physically returned to the Inventory user, that user's cash balance increases; if refund is received by management/bank, the user's physical cash does not change.

Key reports

- User cash balance and settlement history
- Fund requests and fund issues
- Purchases by supplier, product and user, including cash/credit/payment status
- Stock Ledger: stock in / stock out / transfer / return / adjustment / running quantity
- Department issues and acknowledgements
- Supplier Ledger: purchases, purchase returns, payments, credits and outstanding
- Low-stock and out-of-stock alerts

18. Staff Shifts & Attendance Device Integration

24/7 scheduling with reviewed attendance before payroll



Attendance drives Salary; completed doctor services drive Commission separately

Shift configuration

Management first configures department-wise shifts, including start/end time, grace time, late rule, early-exit rule, break, weekly off and overnight behavior. Staff are then assigned to a department and shift.

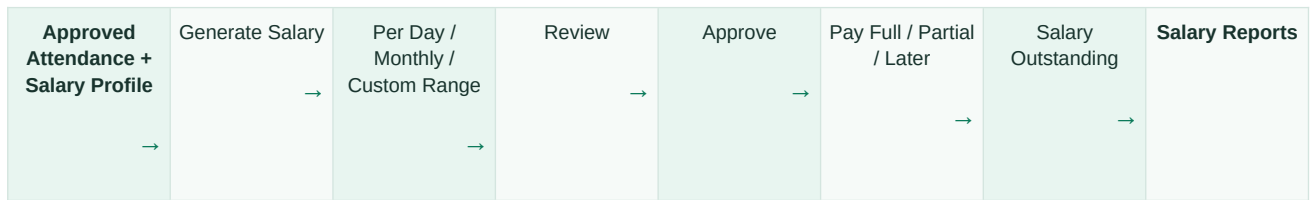
Device integration

Biometric / Device	Raw Punch Logs	Employee Code Mapping	Shift Matching	Normalize Exceptions	Admin / Super Admin Review	Approved Attendance	Salary Engine
→	→	→	→	→	→	→	

- The adapter is vendor-neutral and can later use API, SDK, LAN sync or file import.
- Night shifts crossing midnight remain one shift instance.
- Duplicate punches and missing checkout are normalized before payroll.
- Admin/Super Admin can correct attendance, but every change stores Original Value, Corrected Value, Reason, Changed By and Changed On.
- If attendance changes after salary generation/payment, the system flags recalculation or creates an adjustment; paid payroll is not silently rewritten.

19. Salary Payroll

Independent from Doctor Commission



Salary Payroll calculates only salary-related items. Commission is not included in the salary formula or salary outstanding balance.

Salary component	Treatment
Base Salary	Configured per staff profile / salary basis.
Absence	Deduction according to configured attendance policy.
Late / Early Exit	Deduction according to configured rule/grace policy.
Allowances / Additions	Included when configured/approved.
Adjustments	Explicit positive or negative adjustment with reason.
Payment	Can be full, partial or later; generation does not equal payment.

Independent payout timing

Management may generate a salary statement today and pay it later. A doctor's commission can be generated and paid on a completely different date.

20. Doctor Commission

Service-based accrual and separate payout

Completed Eligible Service	Doctor Attached	Discount Applied	Commission Accrual	Generate Commission	Day / Month / Custom	Approve	Pay Full / Partial / Later	Commission Outstanding
→	→	→	→	→	→	→	→	

Commission rule type	Example
Fixed per service	PKR 900 per completed consultation
Percentage	15% of an eligible procedure amount
Service-specific	Different rule for consultation, ECG, procedure, etc.
Doctor-specific	A doctor-specific rule can override a department default
Commission base	Default: Net After Discount; optional contract override: Gross Service Amount

- A service line can be included in only one generated commission statement, preventing duplicate payout.
- Refund/cancellation after commission generation creates a linked commission reversal/adjustment rather than deleting history.
- Salary outstanding and commission outstanding are never automatically merged.

21. Reporting & Management Accountability

Operational and financial visibility with user-level traceability

Report Family	Examples
Front Desk / Billing	Appointments, advances, OPD/OBS/ER billing, admission collections, cash vs online, discounts/refunds, outstanding, user settlement.
Admission	Planned admissions, active admissions, bed occupancy/transfers, payment request status, medication mode, Pharmacy requests, discharge pending by clearance.
Inventory	Fund request/issue, purchase/receipt, Stock Ledger, Supplier Ledger/outstanding, department issue/return, transfers, adjustments, stock levels, user cash balance and settlement.
Staff 360° / Attendance / Salary	Individual staff profile, employment/shift history, attendance exceptions/corrections, salary profile, generated salary, deductions, paid, partially paid, outstanding and salary history.
Doctor Commission	Service-wise accrual, generated statements, paid, partially paid, outstanding, reversals/adjustments.
Pharmacy	Medicine purchases/receipts, Stock Ledger, Supplier Ledger, batches/expiry, transfers/returns/adjustments, dispensing, Pharmacy invoices/payments/refunds, user Balance Sheet and settlement.
Management	Cross-portal summaries, cash custody, pending settlements, operational alerts and accountability.

Transactional reports use From Date + To Date, quick presets and report-specific filters. PDF, Excel and Print outputs respect the active filters and use the hospital identity from Hospital Profile.

Actual user attribution

Reports and transaction records show the real logged-in actor, not a generic "Admin" label: Collected By, Approved By, Purchased By, Dispensed By, Paid By, Refunded By, Settled By, Received By, Changed By, etc.

22. Worked Example - OPD Appointment

How booking, advance, final billing and doctor share connect

Step	Example outcome
Appointment	Temporary self-pay patient books Cardiology consultation for PKR 5,000.
Advance	Billing user collects PKR 2,000 and generates receipt.
Check-in	Appointment converts to OPD encounter.
Additional service	PKR 1,500 is added during the encounter.
Final services	PKR 6,500 total.
Advance adjustment	PKR 2,000 already received is applied.
Remaining Hospital balance	PKR 4,500 collected/left outstanding as applicable.

Discount / commission example

Calculation	Amount
Gross service	PKR 5,000
10% discount	- PKR 500
Net eligible service	PKR 4,500
Doctor commission @ 20% of net	PKR 900
Hospital remaining share	PKR 3,600

23. Worked Example - Admission

Advance, hospital services, Pharmacy bill and discharge

1. Planned Admission estimate is PKR 100,000. Admission sends the payment request to Billing.
2. Billing collects PKR 40,000 advance. Admission sees "Partially Paid / Advance Received" but does not receive cash.
3. The patient arrives, an actual bed is assigned and the admission becomes Active.
4. Hospital services and bed/procedure charges accumulate to a final Hospital Bill of PKR 135,000.
5. Medication Mode is HOSPITAL MANAGED. Pharmacy receives an PKR 8,000 medicine request but actually dispenses PKR 7,200; Pharmacy invoice is PKR 7,200.
6. The hospital advance is adjusted against the Hospital Bill, leaving PKR 95,000 hospital balance for Billing.
7. Hospital Bill and Pharmacy Bill are cleared independently.
8. When Clinical Ready + Hospital Clearance + Pharmacy Clearance are all true, Admission performs Final Discharge.

If Medication Mode changes

If the patient starts SELF and later requests hospital-managed medicine, only future/new Pharmacy requests follow the new mode. Privately purchased medicines never become Pharmacy charges.

24. Worked Example - Inventory Petty Cash & Settlement

How purchase spending appears in a user Balance Sheet

Inventory user cash	Amount
Opening petty cash	PKR 50,000
Additional approved fund	PKR 20,000
Cash purchases	- PKR 42,000
Approved cash expense	- PKR 3,000
Expected cash	PKR 25,000
Physical cash	PKR 25,000
Variance	PKR 0

The exact Inventory user submits the settlement. Inventory retains supplier purchase history plus a Supplier Ledger for purchases, returns, payments, credits and outstanding balances. The Stock Ledger records every IN/OUT movement. If management pays a supplier directly by bank, the supplier ledger/payment is updated but the Inventory user's physical cash does not change.

25. Worked Example - Salary & Commission

Two separate liabilities, two separate payout decisions

Salary statement

Salary calculation	Amount
Monthly base salary	PKR 60,000
Absence deduction	- PKR 2,000
Late / early deductions	- PKR 1,000
Adjustment	+ PKR 500
Generated Salary	PKR 57,500

Doctor commission statement

Service	Qty	Rule	Commission
Consultation	20	PKR 900 each	PKR 18,000
Procedure	2	15%	PKR 6,000
Total			PKR 24,000

Management may pay PKR 10,000 commission today and leave PKR 14,000 commission outstanding while the doctor's Salary remains completely separate. Salary may be paid on another date and may also be partially paid.

26. Controls, Safeguards & Financial Integrity

Rules that prevent silent history changes or double counting

- Admission cannot collect hospital patient cash; Billing owns all hospital collections.
- Pharmacy owns medicine stock and Pharmacy invoices; HMS owns hospital services, hospital billing and admission.
- Every cash user settles only their own unsettled transactions.
- Online/card/bank receipts do not inflate physical cash.
- Settlement transfers custody; it does not create new income/expense.
- Refunds, reversals and adjustments link back to originals instead of deleting completed history.
- Already-settled transactions cannot be settled again.
- A completed doctor service cannot be included in two generated commission statements.
- Attendance corrections preserve original and corrected values with reason and actor.
- Paid salary/commission figures are not silently changed; later changes create recalculation flags or adjustments.
- Super Admin accounts remain protected from Admin-level alteration.
- Exports do not fabricate hospital legal/contact details; unconfigured data stays explicitly unconfigured.

27. Client Configuration Decisions

Values to confirm during implementation - not missing workflows

Configuration item	Decision to confirm
Attendance deductions	Exact grace minutes and late / early-exit deduction formulas by staff/shift.
Leave / overtime / holidays	Rules for approved leave, overtime and public holidays.
Payroll statutory items	Tax, loans, statutory deductions or benefits if required in scope.
Appointment policy	Cancellation, no-show, reschedule and advance refund rules/approval thresholds.
Manual discount control	Who can discount and which amount/percentage requires higher approval.
Doctor commission contract	Default Net After Discount; specific doctors may use Gross if contract requires.
Settlement cadence	Daily default; shift/custom-period closing can also be enabled.
Panel claims	If insurer receivables/claim submission are required beyond panel discounts and billing reports, detailed workflow can be added.
Attendance device	Final device/vendor connection method: API, SDK, LAN sync or import.

No architecture blocker remains

These are configurable business-policy values. The ownership, hand-offs and financial flows are already defined.

28. Portal-by-Portal Client Summary

What each team will use every day

Portal	Daily users see / do
Super Admin	Hospital-wide dashboard, setup, users/staff, Staff 360° profiles/reports, shifts/attendance, Salary, Commission, finance/cash control, settlements, panel management, reports and integration oversight.
Admin	Same normal hospital-management functions as Super Admin including Staff 360°/attendance/payroll/commission reporting, with protected Super Admin account restrictions.
Front Desk / Billing	Appointments, temporary/panel intake, OPD/OBS/ER, hospital invoices, all hospital collections, admission payment queue, refunds/discounts, My Balance Sheet, My Settlement.
Admission	Planned admissions, active stays, beds/transfers, hospital services, Billing payment requests, medicine mode, Pharmacy requests/status, dual clearances and discharge.
Inventory	Fund requests, petty cash, suppliers, purchase/receipt, Stock Ledger, Supplier Ledger, department issue/return, transfers/adjustments, alerts, My Balance Sheet and My Settlement.
Pharmacy Super Admin	Pharmacy users/settings, medicine/stock governance, finance, settlements, reports and integration monitoring.
Pharmacy Manager	Medicine procurement/receiving, batches/expiry, Medicine Stock Ledger, Supplier Ledger, FEFO, transfers/returns/adjustments, HMS request handling, stock alerts, staff supervision and settlement review if authorized.
Pharmacy Sales / Dispensing	Retail/HMS dispensing, Pharmacy invoice/payment, returns/refunds as permitted, My Balance Sheet, My Settlement.

29. Design Validation & Reference Patterns

The solution follows proven workflow separation while preserving CHSS-specific rules

The v6.3 architecture was reviewed against common patterns from hospital and ERP platforms including Bahmni, OpenMRS O3, Odoo HR/Payroll & Expenses, and Frappe/Marley Health. These references were used only to validate concepts such as appointments vs encounters, ward/bed states, attendance normalization before payroll, controlled expense approval and cross-module hand-offs.

Reference pattern	How CHSS adapts it
Integrated clinical/billing/stock hand-offs	CHSS keeps strong hand-offs but separates Pharmacy as its own connected system.
Appointments, queues and active visits	CHSS separates booking, check-in/encounter, planned admission and active admission states.
Attendance/work entry resolution before payroll	CHSS uses Raw Punch -> Processed Attendance -> Approved Attendance -> Salary.
Expense submission and approval	CHSS uses fund request/approval plus exact-user Balance Sheet and settlement.
Patient registration conventions	CHSS intentionally keeps only Panel Patients as reusable permanent masters; self-pay patients remain temporary case identities.
ERP stock/supplier ledgers and batch control	CHSS uses immutable movement ledgers for general inventory and Pharmacy inventory, with supplier outstanding only where credit purchases exist.

30. Client Walkthrough - Core Understanding

What will be true when the system is implemented

- Each portal has a clear operational owner and only performs its designated work.
- Hospital money enters through Front Desk / Billing; Admission never receives hospital cash.
- Every authorized cash user can see their own balance and submit their own settlement.
- Inventory spending is funded and traceable through exact-user cash accountability, while Stock Ledger and Supplier Ledger preserve quantity and supplier-financial history.
- The standalone Pharmacy controls medicines and returns a separate Pharmacy bill/clearance to Admission.
- Admissions close only after clinical readiness plus both Hospital and Pharmacy clearances.
- Staff records, login accounts, attendance, salary and commission remain logically separated but connected.
- Attendance device data is reviewed before it affects Salary Payroll.
- Doctor Commission is earned from actual eligible services and can be generated/paid independently from Salary.
- Management reporting always identifies the real user responsible for each action.

Recommended approval statement

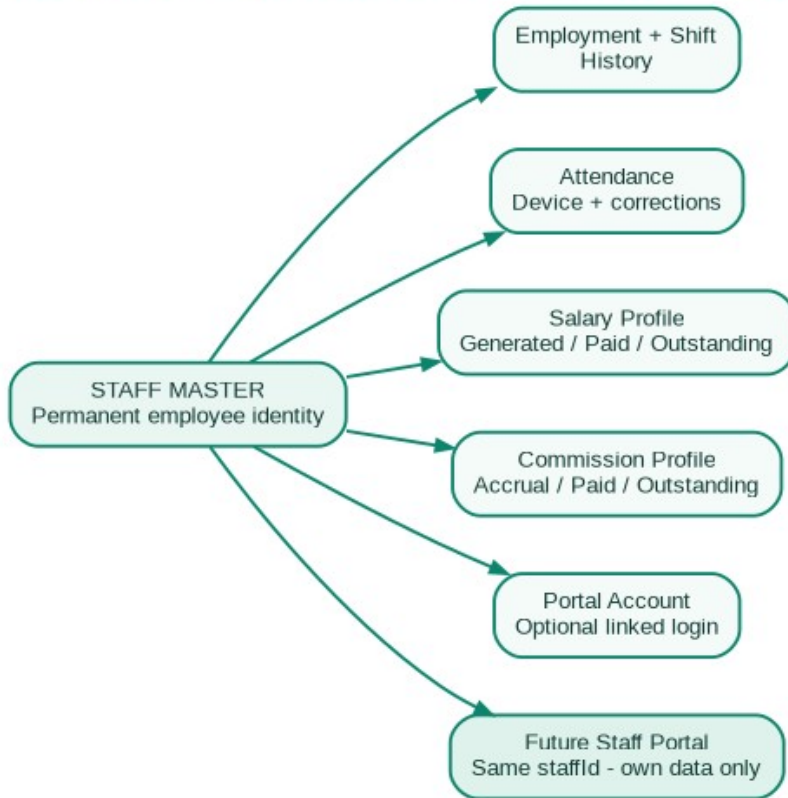
This document can be used as the client walkthrough and functional sign-off basis before final implementation configuration and backend/database integration.

31. Staff 360° Profile & Individual Staff Reporting

One employee identity, complete workforce history, optional future self-service

Every employee - doctor, nurse, technician, receptionist, billing user, admission staff, inventory staff, peon or support worker - has one permanent Staff Master record. Super Admin and Admin open that staff record as a 360° profile instead of searching separate disconnected reports.

Staff 360° Profile and Future Staff Portal



Profile tab	Client-visible information
Overview	Employee ID, category, department, designation, contact, joining date, employment status.
Employment & Shift	Current/historical department, designation, shift, duty hours, weekly off and effective dates.
Attendance	Daily/period attendance, late, early exit, absence, leave, device mapping, corrections and reasons.
Salary	Individual salary rules, calculation breakdown, salary slips, generated, paid, partial and outstanding.
Commission	For eligible staff/doctors: service rules, accrued amount, generated statements, payments and outstanding.
Portal Access	Linked portal username/status, or No Portal Access.

Future Staff Portal

If CHSS later launches a Staff/Doctor Portal, it will link to the same staffid. The employee will see only their own permitted profile, attendance, salary slips and commission statements; no duplicate staff record will be created.

32. Staff-Specific Salary & Commission Configuration

Shared templates in Setup; actual values on the employee profile

Hospital Setup stores reusable policy types and calculation methods, not one global salary or commission amount for everyone. During Add/Edit Staff, management assigns the values that apply to that specific employee and can maintain effective-dated changes when contracts or shifts change.

Shared setup / template	Assigned per individual Staff profile
Salary basis types	Monthly / Per Day / other approved basis and the employee's actual salary rate.
Deduction methods	That staff member's grace, absence, late and early-exit values/slabs where applicable.
Allowance / deduction component types	Only the components applicable to that employee and their amounts/rules.
Commission calculation methods	Doctor/service-specific percentage or fixed amount, commission base and effective dates.
Shift templates	The employee's actual department/shift assignment and effective period.

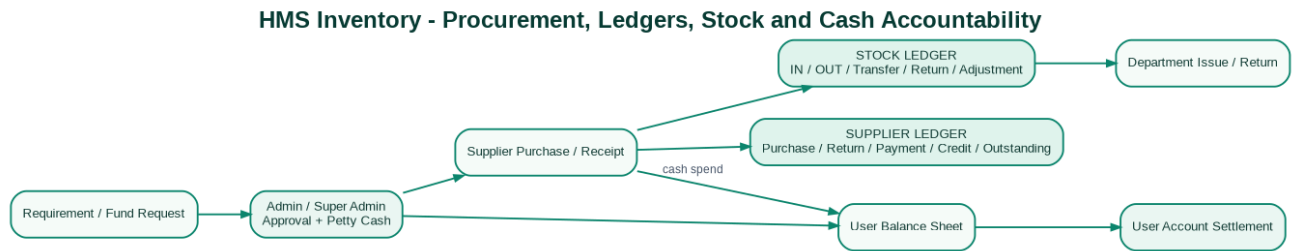
Salary generation and Commission generation remain separate. Management can generate/pay one without generating or paying the other. Attendance affects Salary; actual eligible completed services drive Commission.

Audit requirement

Any change to salary, deduction or commission terms must preserve who changed it, when it became effective and the previous value. Already-paid periods are adjusted/reversed rather than silently rewritten.

33. General Inventory - Ledgers, Procurement and Cash Flow

A full inventory control flow, not just a purchase screen



Inventory component	What it controls
Supplier Master	Supplier identity, contact, terms/status and references.
Purchase / Receipt	Supplier, products, quantities, rates, invoice/reference, payment method and received stock.
Stock Ledger	Every IN/OUT/transfer/return/adjustment with running stock quantity and actor/reference.
Supplier Ledger	Purchases, returns, payments, credits and outstanding balance where credit purchasing is allowed.
Department Issue / Return	Request, approval, issue, acknowledgement, return and department-level movement history.
Transfers / Adjustments	Location-to-location transfers and controlled positive/negative adjustments with reason.
Cash / Settlement	Inventory user's petty cash/advance, cash purchases/expenses, physical cash, variance and own settlement.

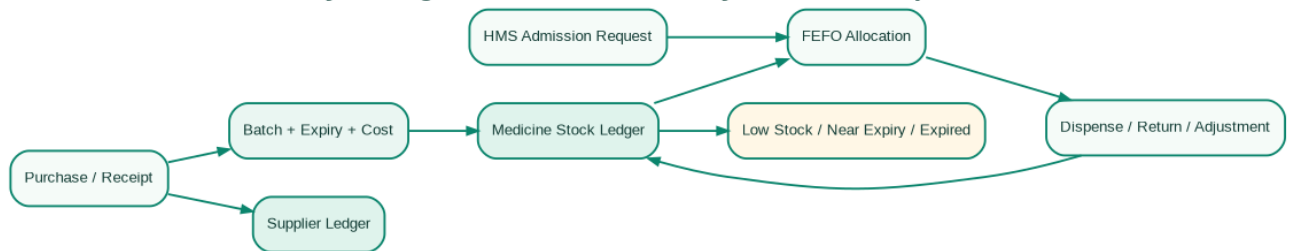
Cash ownership rule

If Admin/Super Admin pays a supplier directly by bank, the supplier/payment and stock records update, but the Inventory user's physical cash does not change. If the Inventory user pays cash, only that user's Balance Sheet is reduced.

34. Pharmacy Manager - Full Medicine Inventory Control

The standalone Pharmacy uses the same ledger discipline, with batch/expiry and dispensing controls

Pharmacy Manager - Medicine Inventory and HMS Request Control



Manager function	Operational rule
Medicine & Supplier Masters	Medicine identity, unit, reorder parameters, supplier terms/status and references.
Purchase / Receipt	Receive medicine against supplier/reference with cost, batch and expiry.
Medicine Stock Ledger	All batch-aware receipts, dispensing/sales, returns, transfers and adjustments with running quantity.
Supplier Ledger	Purchases, returns, supplier payments, credits and outstanding where credit is used.
Batch / Expiry / FEFO	Non-expired stock is allocated using FEFO subject to clinical/operational constraints.
HMS Admission Requests	Accept / partial / reject; dispense actual quantity; create separate Pharmacy invoice and clearance.
Stock Alerts	Low stock, near expiry, expired stock and required action queues.
Staff Oversight	Monitor Pharmacy Sales/Dispensing staff transactions, cash balances and settlements where authorized.

The HMS never becomes the owner of medicine stock. It receives request/dispense/invoice/clearance status through integration references; Pharmacy remains the medicine inventory and Pharmacy billing source of truth.

35. Formula Snapshot for Client Understanding

The detailed Formula Rule Book remains separate; these are the key business equations used in discussion

Area	Core calculation
Service after discount	Net Service Amount = Gross Service Amount - approved discount.
Doctor Commission - default	Commission = Net Eligible Service Amount × staff/service commission %. Fixed-per-service rules are also supported.
Hospital Share	Hospital Share = Net Service Amount - Doctor Commission.
Admission Hospital Due	Final Hospital Bill - all Hospital payments collected/applied by Front Desk/Billing.
Generated Salary	Period Base + approved salary additions - attendance deductions - other approved deductions ± adjustments.
Salary Outstanding	Approved Generated Salary - Salary Payments Applied.
Commission Outstanding	Approved Generated Commission - Commission Payments Applied.
Billing Expected Cash	Opening Float + cash collections + cash received - cash refunds - authorized cash expenses.
Inventory Closing Qty	Opening + receipts + returns/transfers in + positive adjustments - issues - supplier returns - transfers out - negative adjustments.
Supplier Outstanding	Opening Outstanding + credit purchases - supplier payments - purchase returns - approved credits.

Important

Formula parameters such as payroll divisor, grace minutes, deduction slabs, commission percentages, approval limits and settlement cadence remain configurable according to CHSS policy and staff contracts.

36. Client Sign-Off Summary

What the approved solution will guarantee

- Every hospital operation has a clear portal owner and every financial/operational action records the actual logged-in user.
- Super Admin and Admin can review every Staff 360° profile including attendance, salary and commission history; protected Super Admin account safeguards still apply.
- Front Desk/Billing owns all HMS patient cash collections, appointments, advances, partial/final admission payments and its own cashier settlement.
- Admission is non-cash: it manages planned/active stays, bed/services, SELF vs HOSPITAL MANAGED medicines, Billing requests and dual discharge clearances.
- General Inventory maintains Supplier Master, Supplier Ledger, Stock Ledger, purchase/receipt, issues/returns/transfers/adjustments, petty cash, Balance Sheet and exact-user Account Settlement.
- Standalone Pharmacy owns medicine purchases, supplier/stock ledgers, batches/expiry/FEFO, dispensing, Pharmacy invoices, Pharmacy cashier settlements and HMS medicine-request fulfillment.
- Salary and Doctor Commission are separate generation, approval, payment, outstanding and reporting streams; employee-specific rules live on Staff profiles.
- Biometric/device attendance is normalized and reviewed before it affects payroll; corrections preserve original/corrected values, reason, actor and time.
- Only Panel Patients are reusable permanent patient masters; self-pay visitors use temporary case identities while invoices/receipts/transaction snapshots remain retained.
- All reporting supports period filters and exact-user accountability; PDF/Excel/Print outputs use the configured Hospital Profile identity.

Functional sign-off basis

This Full Client Edition is the client-facing explanation of Source of Truth v6.3 and can be used alongside the portal PDFs, workflow/diagram PDF and Formula Rule Book during final implementation sign-off.

CH Sharif and Saeed Hospital - Hospital Management System

Full Client Functional Walkthrough | Source of Truth v6.3

Prepared by iSysware Software Solutions